



Induction of labour?

Here's your guide





Message from @birth-hood

Hi,

I'm Leanne, mum of two, (and a cocker spaniel, Ron) Doula, Perinatal Yoga teacher and Birth Trauma Resolution practitioner, and obviously a hypnobirthing nerd!

When you become pregnant, for the first, or the fifth time, your mindset changes instantly, as well as being excited, it can be overwhelming and full of uncertainty. Especially if you're told you need to be induced... but what does this mean? What options do you have? How will this be done? Oh the questions!!

It can be daunting when you don't have the answers to this, so here is a guide to help you understand the processes, what your rights and options are as well as how to make it more positive if you choose to accept one.

If you have any questions, please don't hesitate to contact me.

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Leanne xx

What is Induction of labour?

Induction of Labour (IOL) is defined as an intervention designed to artificially initiate labour (uterine contractions leading to progressive effacement and dilatation of the cervix), and birth of the baby. Basically, get your body ready to birth your baby. This may include women and birthing people with membranes still intact or women and birthing people with ruptured membranes who are not in labour.

Induction of labour is indicated when it is agreed that there is a higher probability of a healthier outcome for mother and birthing person and / or baby to induce birth than if the pregnancy were to continue.

Induction of labour should only be considered when vaginal birth is felt to be the most appropriate route.

Currently the NICE guidelines state that induction should be offered between 41 and 42 weeks of pregnancy. Many people are led to believe that this is because the chance of stillbirth after this time is high. However, the research into this has shown us that if the chance of stillbirth increases at all (studies show various different results) then the chance is small at any point of pregnancy. Less than 2 babies per 1000 births. A recent Cochrane review estimated that 426 people would have to be induced between 41 and 42 weeks of pregnancy to avoid one stillbirth.



Informing Decision:

Before the induction process begins, women and birthing people should be informed about the clinical indication of the induction and the associated risks and benefits. Women and birthing people should also be informed of the arrangements for support and pain relief. Written information to support their decision making should be provided. Alternative options should a women and birthing people chose not to have an induction should be explained.

Always give yourself time to make a decision, get a second opinion, ask for the absolute statistics.

Here's some things to consider when opting for an offer of induction:

- Your hospital stay may be longer than with a spontaneous labour
- Vaginal examinations to assess the cervix are needed before and during induction
- Your choice of place of birth will be limited, as they may be recommended interventions (for example, oxytocin infusion, continuous fetal heart rate monitoring and epidurals) that are not available for home birth or in midwife-led birth units
- There may be limitations on the use of a birthing pool
- There may be a need for an assisted vaginal birth, with the associated increased risk of obstetric anal sphincter injury
- Pharmacological methods of induction can cause hyper-stimulation
- An induced labour may be more painful than a spontaneous labour
- Induction may not be successful, and how this would affect the woman and birthing person's options.

NICE guidelines state that induction has a large impact on the birth experience, it is more likely to result in an epidural and assisted delivery. It is a big decision and always up to the birthing person.

Alternatives could be waiting or offering increased antenatal monitoring including scans, depending on the reason induction is suggested. It is also your right to request an elective caesarean if you would rather go down that route rather than attempt an induction. It should be clinically justified, there should be a reason that it is safer for baby to be born than remain in the womb and that induction is the way the birthing person would like to do that.

1 in 3 labours are induced, 70% for dates alone. It is important to get informed about induction as there is a good chance you will be offered one at some point in your pregnancy. Be aware!

Informing Decision:

Before commencing an induction of labour, your provider will assess your readiness using a calculation called the Bishop score.

This is a calculation used to predict how close you are to labour. Your healthcare provider will examine your cervix and determine your Bishop score based on changes in your cervix and the position of your baby's head. Your final score can help determine if induction will lead to a successful vaginal birth. Your score can range from zero to 13, with zero meaning you're not ready for induction and 13 indicating a better chance for successful induction. As your body prepares for labour, your cervix softens, thins and opens up. This can happen several weeks or several days before your baby is born. Your Bishop score rates how ready your cervix is for labor by looking at how it has changed.

Your provider uses these five factors to calculate your Bishop score:

- Dilation of the cervix: Dilation refers to how open your cervix is. A fully dilated cervix is about 10 cm. Dilation is measured with fingers, then translated to cm.
- Effacement of the cervix: Effacement means how thin or short your cervix is. Fully effaced (or 100% effaced) means your cervix is "paper-thin."
- Consistency of the cervix: This refers to how tough your cervix is. A softer, more flexible cervix is more likely to dilate. Think of it as comparing the hardness of the tip of your nose to the softness of your lips.
- Position of the cervix: The position of your cervix moves closer to your birth canal as labour nears. This is referred to as being anterior. If your position is posterior, you are further away from labour.
- Fetal position (or fetal station): This measures the position of your baby's head in relation to the ischial spine (a place in your pelvis). Healthcare providers give a rating based on if the head is above or below that point in the pelvis. A positive number means your baby's head has descended into the birth canal, and a negative number means your baby's head is still high.

Each factor is given a score. Then, the five individual scores are added to get your final Bishop score. A higher Bishop score means inducing labor is likely to be successful.

Dilation of cervix	Position of cervix	Effacement of cervix	Consistency of cervix	Fetal position	Number of Points
Closed	Posterior	0% - 30%	Firm	-3	0
1-2 cm	Middle	40% - 50%	Medium	-2	1
3-4 cm	Anterior	60% - 70%	Soft	-1, 0	2
5+ cm	--	80%	--	+1, +2	3

Use the BRAIN acronym

The BRAIN acronym is an important decision making tool that everyone should know when accessing maternity care. It can be used to make decisions regarding scans, treatment, monitoring, tests, interventions and birth plans and integral during induction decision making. Having this tool and using it alongside your own research into your options can transform the way you feel about it all. Knowing exactly why something is offered, what other options there are and all about it makes it a totally different experience to feeling confused and out of control and not able to be confident in your decisions. You are in charge here and when you have all the information you need you are able to make the decisions that are best for you. You've got this!

Benefits

Find out the benefits of the induction. What is the aim here? What will it achieve? WHY is it being offered? Is it 'just in case' or is there a known reason to do it in your case specifically or is that just policy? For example an induction may be suggested due to the birthing parents age as that's what the policy is there.

Risks

What are the risks of the induction? How safe is it for you? How safe is it for baby? Does it increase the chance of needing any other tests, treatments or interventions? For example, if an induction is suggested due to baby or parent being unwell, what are the risks relating to their condition, how well are they expected to cope?

Alternatives

Is there anything that could be offered instead? Instead of an induction is there monitoring that could be done? When you know what all the options are then you can get informed on them all and make your choice.

Instincts

What does your instinct tell you about it? Instinct is a really important thing to consider in pregnancy, birth and parenthood. Often the parent will know that something is not ok with baby before health professionals do. Similarly, they will often feel that all is well with baby despite a professional believing otherwise. So what do you think? What is your gut feeling? Do you feel like all is well and you're happy to continue as you are or do you think the induction might be necessary?

Nothing

What happens if you do nothing instead? Not the induction, not an alternative, just nothing. What is expected to happen then? Is it nothing? Is there a small chance of something? Or is there a big chance of something occurring? It may be that you do nothing for an hour, a day a week or maybe just nothing at all as long as all is well. It's completely up to you.

What is the process?

Stretch & Sweep

A sweep is a form of induction as it aims to encourage the body to go into labour before it would have happened on its own. However, people are not encouraged to be in hospital or anything after having one so if labour begins after that you would go on to labour in your chosen birthplace etc. They are often offered routinely at the end of pregnancy, the gestation that this would happen will vary based on area.

Process:

Via a vaginal exam the midwife or doctor will insert a finger into the cervical opening, if possible, and try to run a finger around the inside to separate the membranes (the sac containing your baby) from the wall of the uterus/ top of the cervix.

If the cervix is not open they can sweep the outside. A sweep aims to irritate the cervix and uterus which releases prostaglandins. Usually a certain amount of discomfort is experienced during a sweep as it is not a gentle procedure.

Evidence: It's impossible to get accurate evidence on sweeps as if someone goes into labour the day after a sweep, it is impossible to know if they would have gone into labour then anyway, since sweeps are only given at the end of pregnancy when labour is likely to happen. The evidence from a Cochrane review that brought lots of studies together showed that the use of sweeps in first time parents did 'not seem to produce clinically important benefits' when used to prevent the need for further induction interventions. However many birthers would like to try it as a first step to see if they can avoid induction. Sometimes this can have positive mental benefits as it can make you feel like you've tried all you can. It is worth remembering at this point that the cervix can change quickly so if you attempt a sweep and find that your cervix doesn't seem to be gearing up for labour then don't be disheartened as you could still go into labour any minute! Get that oxytocin flowing!

Side effects:

- Sweeps can induce discomfort similar to surges/prolonged early labour which can lead to exhaustion and is linked to higher epidural use and further interventions.
- They can cause bleeding and discomfort to the cervix which can impact oxytocin even if it turns out to be harmless.
- In some cases the membranes (securing your waters) can be broken and this would then begin the clock on your local policy of how long to leave labour to begin before stepping in to induce or offer caesarean. It is always up to you how long you would like to leave it or if you are happy to go with the recommendation but it can be more decisions to make and can impact oxytocin.

What is the process?

Pessary:

The pessary' is a synthetic prostaglandin inserted next to the cervix to soften it ready for labour. In some trusts this is offered as an outpatients procedure but in most areas this is something that you have done in hospital on a ward. Depending on the method of this, this may be left in for 6 hours or 24 hours and there can be multiple tries. This part of induction can be very long so make sure you are prepared for a wait and have brought entertainment with you. It can also increase the likelihood of overstimulation of the uterus that can be very intense.

In some cases you may be offered a **Foley balloon Catheter** instead, the balloon induction is a mechanical form of induction of labour and is the only method used for outpatient induction of labour at present. This is because mechanical methods of induction of labour have the least chance of overstimulating the uterus and causing too many contractions (uterine tachysystole).

The procedure involves a soft silicone tube also known as a catheter being inserted into the neck of your cervix. It has a balloon near the tip and when it is in place the balloon is filled with a sterile saline (salt water) fluid. The catheter stays in place for up to 24 hours, with the balloon putting gentle pressure on your cervix. The pressure should soften and open your cervix enough to start labour or to be able to break the waters around your baby.

Inserting the pessary or balloon into the cervix is uncomfortable, and can sometimes be painful. There is a small risk of infection and if this is suspected you will be re-assessed and your plan of care should then be adapted according.

What is the process?

ARM- Artificial Rupture of Membranes

If your cervix is dilated enough, your water can be broken obviously if you consent to this. This is done using a knitting needle type tool, called an amnio hook, the vaginal exam is the main part of this which may be uncomfortable. The aim of this step is to have baby's head make contact with the cervix and help dilation along. Labour can become more uncomfortable after the cushioning of the waters has been removed so have a think about this in your birth plan. Include how you would like to manage your comfort levels.

Risk factors associated with ARM include:

- Maternal or neonatal infection
- Umbilical cord prolapse
- Abnormal Fetal heart rate
- Disruption of an occult placenta previa or vasa previa with subsequent maternal haemorrhage.

Syntocinon

If no contractions start then 'the drip' will be offered. Syntocinon is an artificial oxytocin which will cause contractions. it is inserted into your body via a cannula in your hand.

Unfortunately not the same as natural oxytocin in that it doesn't make you feel happy and full of love as it doesn't enter your brain, it affects only the uterus and does this very quickly and with intent. Monitoring your baby regularly or continuously is often suggested when using the drip to keep an eye on how baby is coping with the surges, as it can cause the baby to become more stressed.

Contractions from syntocinon are said to be very intense and have no real breaks in between them as spontaneous surges would. This is very normal and not a reflection of how well you can cope with labour. Don't be disheartened if you would like stronger pain relief for this.

Syntocinon can increase likelihood of analgesia, can increase PPH post birth, the blood and oxygen supply reduce compared to physiological labour, this increasing the risks of hypoxia (low oxygen) for your baby, it can reduce your mobility that can increase likelihood of ventouse or forceps birth.

FAQ

What happens on the day of the induction?

A decision may be made to change your method of induction on the day of your induction. After a vaginal examination, changes to your cervix may mean that another method of induction is more appropriate. Your doctor or midwife will discuss this with you. If there has been a change to your cervix, then remember you always have to consent to any procedure, so if your body is showing signs of spontaneous baby, and the risks to your and/or baby are stable, you may choose to hold off, this is your choice, remember your BRAIN.

How long will it take?

Once the induction has started, you will be monitored regularly. You can walk around but you will be advised not leave the hospital. Your cervix is assessed regularly to check its progress (remember this can increase risk of infection). Induction is not a quick process. Once it has started, it may take more than 24 hours until your baby is born. If your cervix needs to be primed, it may take two days or more. Be prepared for a wait- bring distractions.

Can I change my mind after induction has started?

Once induction has commenced, it is expected to continue until your baby is born. Hospitals will not recommend stopping the induction process, however it is ALWAYS your choice and you must consent to each stage.

Will it be more painful than natural labour?

Every labour is different. For some women/birthing people an induced labour is more painful than a labour that starts on its own. As these processes are not physiological and are instead synthetic, the normal 'happy' hormones might not be released so you need to do all you can to help them get flowing. Think, cuddles, music, lighting, smells etc.

You can also try:

- Keeping mobile and use active, upright positions to help your baby into an ideal position for birth
- Heat packs, hot water bottles or massage on your lower back may help relieve any cramping
- Warm bath or shower
- TENS machine
- You can take 1000mg paracetamol every 4-6 hours (maximum four doses in 24 hours).

FAQ

Will it work?

Sometimes induction does not work and you may not go into labour. If this happens and all safe options have been tried, you may need a caesarean.

Make sure you have planned for all eventualities so that you can keep an element of control.

Can my partner stay with me for inpatient induction?

Yes, your birth partners are advised to stay with you throughout the induction process. On some wards there are cozy chairs for the birth partners who wish to stay overnight.

Can I use the Birth Centre?

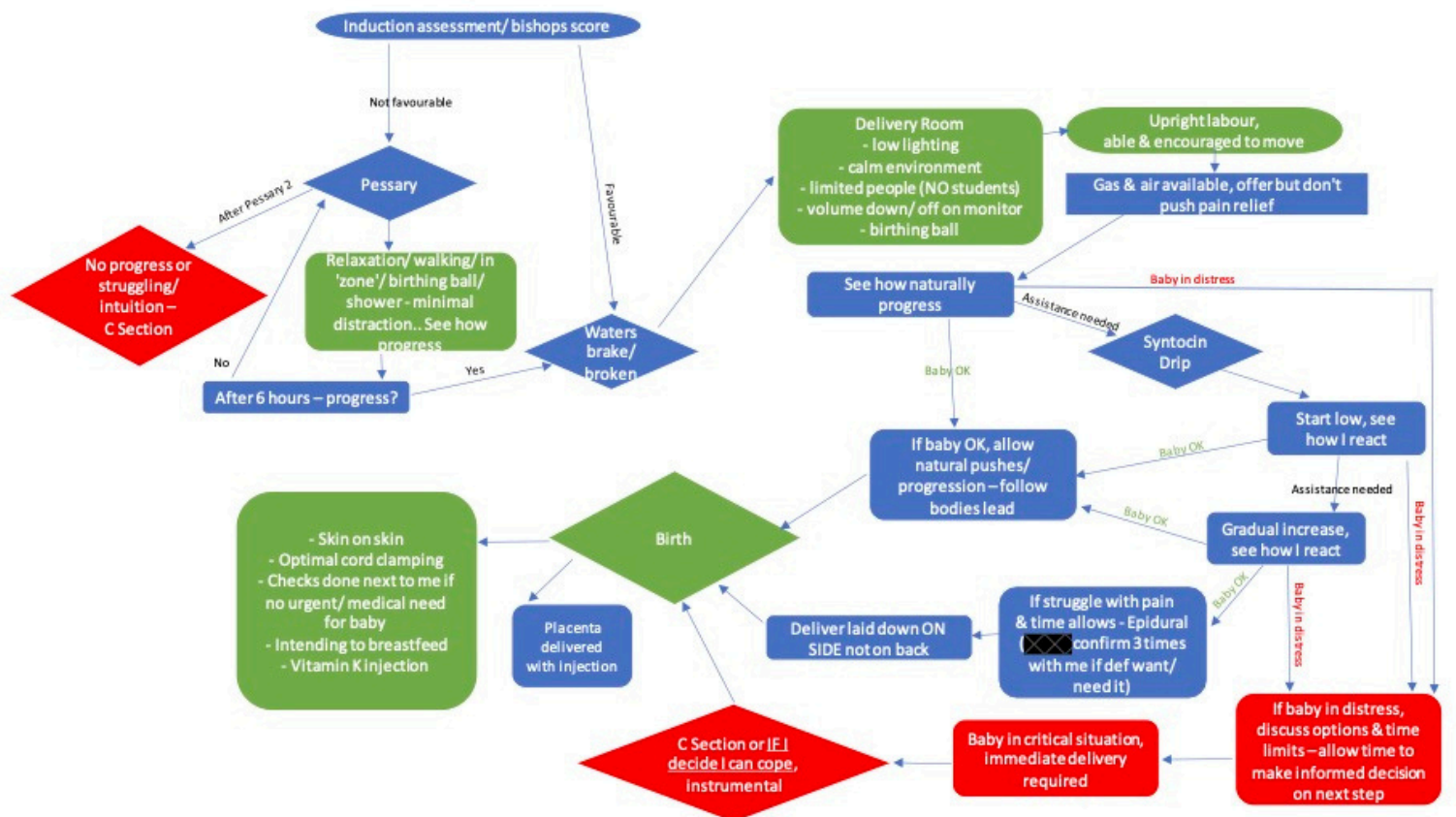
Most women/birthing people having an induction may need medications and monitoring that are only available in the labour ward. However, it is possible to use the Birth Centre if your pregnancy is considered low risk and your induction helps labour to establish within 24 hours of it being commenced. 'Established labour' means that your body has taken over the physiological process of having contractions and your cervix is more than 4cm dilated and progressing. If you are not in established labour, they would recommend you attend the labour ward to be offered artificial rupture of membranes and given medication to help your labour progress. Speak to the Professional Midwifery Advocate to discuss your induction and to voice your wishes and needs for the process.

How many inductions go to unplanned caesarean?

In NHS trusts, around 30% of inductions (1 in 3) go to an unplanned caesarean birth, 50% of inductions (1 in 2) go to a vaginal birth and 20% of inductions (1 in 5) go to an assisted birth (ventouse or forceps).

Please see my 'BIRTH PLANNING GUIDE' below to make sure you are fully prepared for your births, planning for your ideal, induction and caesarean at least!

Here's an example guide to help you make decisions about induction of labour.



Firstly, writing a birth plan is NOT a silly idea or an invitation to be disappointed. Anyone who thinks that way is not yet informed about how bloody important a birth plan is! A birth plan is not a prescription or a set of instructions, its just a clear presentation of what you want! It will help your care providers to know exactly what you want without asking you a million questions but most importantly it will help YOU and your birth partner/s to learn all the options available and know what you want!

Visual Birth Plan Icons: You can find these on www.pinterandmartin.com. These are a great way to have a first look at what all the options are. There are likely to be lots of them that you hadn't considered or have just never heard of before. Find out what they all mean if you're not sure, knowing what all the options are is a great way to start figuring out what you would like to choose!

You are looking to create AT LEAST 3 birth plans. Think about what changes could happen. Eg. Home birth to hospital, spontaneous to induction etc. You would then create birth plans for each scenario with plan A being your ideal birth and plan C being a caesarean birth. Someone planning a Labour ward birth may do a plan A for that ideal birth they are working towards, a plan B for if they decide to have an induction or any augmentation (waters being broken, the drip etc.) and then a final plan C for caesarean. If plan A was a home birth or birth centre birth then there may be a plan B x2 for a change of birth place too!

Think about what you want! What's important to you? What birth have you always thought would be lovely? At this point it doesn't matter what type of birth you think you are limited to, for now you are just getting an idea of the type of birth you would love to have. The next steps will help you find out how you can make this happen or what alternatives there are.

What special circumstances do you need to consider? Find out everything you can about these- Places like Evidence Based Birth, Sara Wickham's website, the NICE guidelines, AIMS. These may be health issues of your own, pregnancy related issues, health issues for baby. Not all of these will affect your birth choices but now is the time to find out which ones do if you have any!

Find out others experiences. For example, If you would love a home birth and you have gestational diabetes, Join a FB group like Home Birth Support Group UK and look for others experiences with GD. This can be a great way of getting some insight into what other people have found useful when making their decisions and the different care provider opinions others have come across. This might show you that it's not as black and white as you may have been led to believe. No matter what it is always your choice.



birth proposal

NOTES TO MIDWIFE

- Name:
- Birth partner's name:
- Homebirth
- Doula: Leanne
- Gender unknown

ENVIRONMENT

- Low lighting
- Aromatherapy
- Galaxy lighting
- Birthy playlist
- TV shows
- Birth ball
- Eye mask
- No mention of time

PAIN RELIEF OPTIONS

DO NOT OFFER PAIN RELIEF

- Breathing
- Water
- Combs/Acupressure
- Massage/counter pressure
- Encouragement/Affirmations
- Cold Flannel/fan
- Essential oils
- Birth ball
- TENS Machine
- Gas & Air
- I am open to other medication if transferred and i request

MONITORING

- Intermittent monitoring
- I do not want vaginal examinations
- Check waters and signs of infections via temperature and baby movements if SROM/PROM
- No offer of induction on due date
- After 40 weeks extra monitoring and open to discussion for next steps at 42w

COACHING

- Please encourage 'breathing baby down'
- Go with body (let go)
- Encouragement and 'you're safe'
- I do not mind being coached to push if necessary. I will communicate if my needs change and therefore no longer require support.
- I do not want to push in between contractions.

BIRTH POSITIONS

- Birth in water
- UFO positions
- (Over ball, side of pool, lunge)
- Leanne to assist if necessary



birth proposal

INTERVENTION

- No intervention unless absolutely medically necessary and discussed with Leanne
- No sweeps
- No cannula just in case
- I do not want assisted delivery unless absolutely no other choice.
- Please avoid the use of an episiotomy.

CATCHING THE BABY

- Dependent on birth baby (in water/out etc)
- I would like help to catch baby and bring to chest, **to help and confirm gender of baby.
- If I birth in water I would like to remain in the water until I specify.

3RD STAGE

- Natural expulsion of placenta (no injection unless signs of PPH or maternal request)
- WAIT FOR WHITE - Optimal Cord Clamping (potential lotus)
- Hemal to cut cord
- Placenta to be encapsulated
- Placenta checked in view- We would like to see it.

GOLDEN HOUR

- No hat when skin to skin unless baby in compromised
- Skin to skin (at least 1 hour)
- ** skin to skin after
- Delay weighing baby (at least one hour)
- Breastfeeding
- Vitamin K via injection
- I consent to NIPE check

NOTES

I wish for my birth partner to confirm the gender of the baby to me.
Please have any discussions with my birth partner/doula in the first instance, for them to relay the information to me.
Please avoid any discussions about me in the room.
Please could these be conducted elsewhere.
Please allow me to move freely and use the toilet facilities as required.

Talk through your options with knowledgeable folks. Leanne or the AIMS helpline would be a good start, they can tell you what they know, any relevant research and help you come up with a list of questions for you to ask your care providers as well as stuff to read up on.

Discuss with your care provider. This may be a midwife or a consultant, do not be disheartened if you don't hear what you would like to hear, you can still chat through your circumstances with a Professional Midwifery Advocate and they can talk you through if what you are after is REALLY out of the question or not. (note, it rarely is!)

Consider what things are super important to you and you would not like to miss out on unless there's an emergency. This may be remaining upright throughout labour and for the birth, skin to skin after birth, avoiding 'the drip' or any other intervention or medication etc. How can you try to make sure that you can have these things? Can they be worked into all your birth plans? Eg. Remaining upright- even in the case of epidural use you can be assisted into various positions that support birth. Skin to skin after birth, can it be arranged that your birth partner holds baby on your chest for skin to skin EVEN after a Caesarean under general anaesthetic? Avoiding 'the drip', this may mean you opt for a caesarean if you would rather. There are so many possibilities we're not always offered, if you aren't sure how you can still incorporate certain things then talk to your instructor!

Create the birth plans for real! Consider everything you've learned about any factors concerning your pregnancy and chats with PMA or midwife etc and bring it all together to create your plan using whichever format suits you best, try to ensure it will be easily read by the care providers and explained by your birth partner.

Create your plan methodically by first considering your background such as how you feel about birth, how many babies you've had, any special medical or cultural requirements.

Then move onto the things that you would like to include in your environment such as low light levels, quiet, who you would like present (eg. Are you happy to have student midwives present? A doula?).

Next consider what you would like for each stage of labour, comfort measures you'd like to have available, what monitoring or interventions you would like to choose or avoid. Make sure to include what you would like your midwives to do, whether you would prefer them in the background or if you would really like the reassurance of having them right next to you when they're in the room. Include how you would like baby to be born, whether you would like direction from a caregiver or not and if you would be happy to accept forceps etc if thought necessary.

Then consider what you would like to happen once baby is here, would you like to find out the sex yourself?

Who will cut the cord? When will it be clamped (if at all)?

What about vitamin K injection for baby? Let those reading know how you would like to feed your baby and what you would like to happen with baby when they are born, would you like uninterrupted skin to skin?

Would you like baby wiped down before you hold them? This is all up to you! Finally, how would you like to birth the placenta? What would you like to do with it after?

Do this with your birth partner ideally so that they understand it all and know the reasons for your choices. At the very least, go through it in detail with them, they will then be able to help relay it to your care providers on the day.

Change your birth plan any time you want, we often learn new things and make new choices. Finally, you can change your mind on the day if you want to!

Have a look at my 'Peanut Ball' guide to see how you can use this to help during birth (especially with the use of epidural etc which is more likely with an induction of labour).



Order in the most important things for YOU.

Things to consider:

Plan A (ideal) Preferences

- Place of birth
- When to go to the birth place.
- Membrane sweeps
- Student midwives/drs
- Use of hypnobirthing
- Eat and drink in labour
- No offer of pain relief
- Any special/cultural requirements
- Monitoring when 'post term' (42 weeks) and during labour (continuous/intermittent)
- Lighting, communication, music, smell, visuals
- Use of pool/bed/positions
- Vaginal examinations
- Artificial rupture of membranes in labour
- Positions for labour
- Pain relief (water, breathing, aromatherapy, massage, comb etc)
- Pharmaceutical pain relief (entinox, pethidine, water diamorphine, epidural)
- Coaching during 2nd stage
- Hands on/off approach
- Third stage (natural or managed)
- What you plan to do with the placenta
- Optimal cord clamping and who to cut the cord)
- Golden Hour (undisturbed, breastfeeding, skin to skin, who cuts the cord)
- Wipe the baby down or not
- How you plan to feed
- Vitamin K

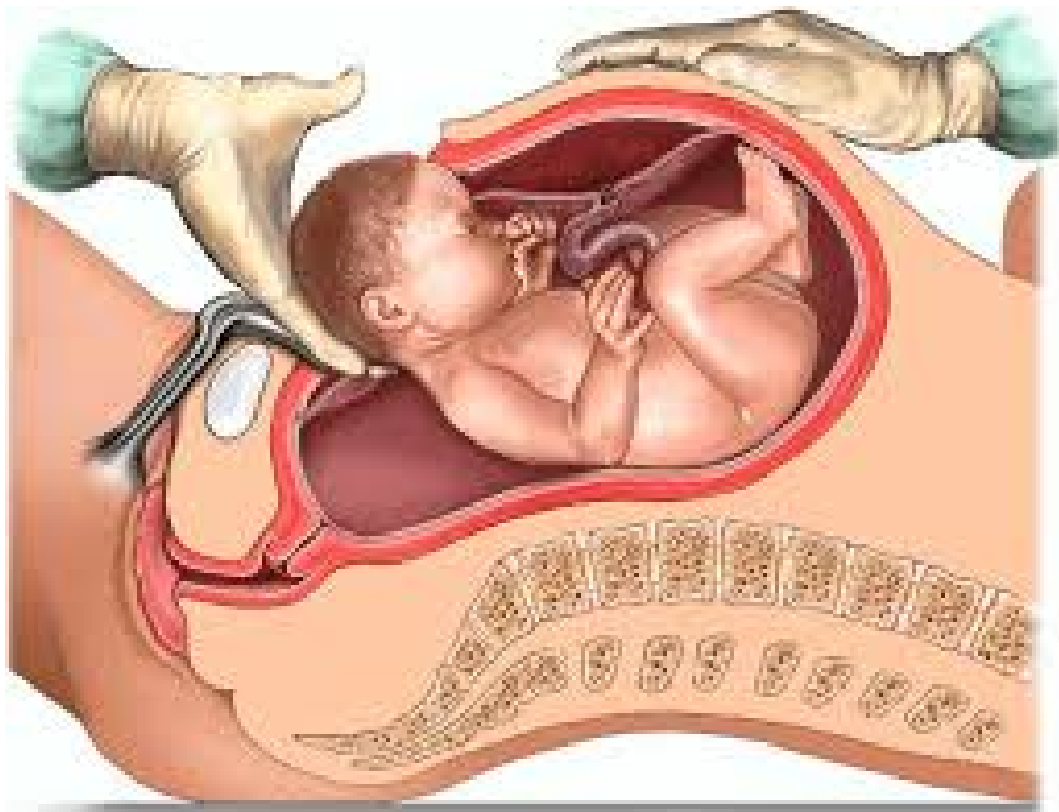
Plan B- Induction/hospital preferences (additions to above)

Induction preferences (when/how)

- Induction by membrane sweep
- Induction for post dates
- Induction of labour does not start after release of membranes
- Vaginal examinations after release of membranes
- Monitoring when 'post term' (42 weeks) and during labour (continuous/intermittent)
- Assisted delivery (incl. forceps)
- Other augmentation of labour (aka the drip)
- Which hand you want cannula

Plan C - Caesarean preferences (Golden hour etc still use above informatin to help you)

- Emergency vs unplanned
- You have to agree and consent- how do you feel?
- Lights
- Scripts
- Music
- Partner (affirmations, touch, cuddles etc)
- Where to place CTG
- Which hand for cannula
- Breathing
- Gentle birth
- Announce gender or not
- Curtain lowered
- Watch birth/hear a description
- Immediate skin to skin
- Optimal cord clamping when and by who
- When to have catheter out
- Post birth support





birth proposal

NOTES TO
MIDWIFE

ENVIRONMENT

PAIN RELIEF OPTIONS

MONITORING

COACHING

BIRTH POSITIONS

Name:
Pronoun:
EDD:
Preferred place of birth:

Birth partner name:
Pronoun:
Contact:



birth proposal

INTERVENTION

CATCHING THE BABY

3RD STAGE

GOLDEN HOUR

NOTES



CAESAREAN

birth proposal

NOTES TO MIDWIFE

ENVIRONMENT

PREFERRED ANAESTHETIC

MONITORING

PRIOR TO BIRTH

DURING THE BIRTH

Name:
Pronoun:
EDD:
Preferred place of birth:

Birth partner
name: Pronoun:
Contact:



CAESAREAN

birth proposal

CATCHING THE BABY

3RD STAGE

DURING RECOVERY

GOLDEN HOUR

NOTES